Virtual Patient Encounter Rubric

Conversational clinical interview only — no physical exam. Scoring: 16 items × 3 pts = 48 total.

3 — Exceeds Expectations2 — Meets Expectations1 — Below Expectations

Criteria	3 — Exceeds	2 — Meets	1 — Below
Section 0 — Introduction			
0.1 Introduction / Rapport Building Did the student introduce themselves and set a comfortable tone before clinical questions?	Introduces self (name, role); greets warmly; makes small talk or asks how patient is feeling; establishes purpose and agenda; confirms patient's immediate concern	Brief introduction, moves quickly into questions without building rapport; establishes a workable focus but misses agenda or patient priorities	No introduction; jumps straight into questioning; begins abruptly or creates confusion
Section 1 — History Assessment			
1.1 General HPI Generic — same for every patient. 10 items: Character, Location, Onset, Duration, Prior history, Radiation, Intensity, Trigger/Improvement, Setting, Social Impact	Asks 7+ of the 10 items	Asks 4–6 of the 10 items	Asks 0–3 of the 10 items
1.2 Case-Specific History Faculty-defined per patient. 3–5 associated symptoms the student should ask about.	Asks about majority of case-specific symptoms (≥70%)	Asks about some (30–69%)	Asks about few or none (<30%)
1.3 Medical History	Asks about 5–6 categories	Asks about 3–4 categories	Asks about 0–2 categories

Generic — same for every patient. 6 categories: PMH, PSH, Medications, Allergies, Social Hx, Family Hx			
Section 2 — Closing / Counseling			
2.1 Differential Diagnosis Discussion Did the student share their thinking about what might be going on? Faculty defines expected differentials.	Discusses 3+ appropriate differential diagnoses with the patient	Discusses 1–2 appropriate differentials	Does not discuss, or discusses incorrect diagnoses
2.2 Next Steps / Workup Did the student explain what tests or studies they'd recommend? Faculty defines expected workup.	Explains all expected next steps (labs, imaging, other tests)	Missing 1–2 key next steps	Does not discuss next steps
2.3 Treatment / Medications Did the student discuss a treatment plan? Faculty defines expected treatment.	Explains appropriate treatment plan including side effects in patient-friendly language	Discusses treatment but components are not appropriate or first-line	Does not discuss treatment
2.4 Diagnostic Convergence NEW Did the student's questioning progressively narrow toward a plausible assessment?	Questions build on each other logically; the student's line of reasoning visibly converges toward the correct diagnosis; closing statements reflect what was gathered	Some evidence of narrowing, but with tangents or gaps in the logical chain; reaches a partial or incomplete conclusion	Questions feel random or disconnected; no visible narrowing; closing does not follow from what was gathered
Section 3 — Organization / Efficiency			

3.1 Organization of Encounter Did the interview follow a logical structure?	Systematic interview — chief concern → HPI → medical history → social/family history → closing. Questions flow logically.	Mostly organized but a few questions out of order or jumping between topics	Interview completely out of sequence, difficult to follow thought process
3.2 Efficiency / Pace Did the conversation flow without unnecessary repetition?	Conversation flows well, no unnecessary repetition, builds on prior answers	Occasionally re-asks something already answered, or has 1–2 unfocused tangents	Frequently re-asks questions, goes in circles, fails to build on information provided
Section 4 — Communication Quality			
4.1 Empathy / Rapport Did the student acknowledge patient concerns and build trust?	Acknowledges concerns; validates feelings; shows warmth; uses attentive, nonjudgmental, culturally responsive language; patient appears heard	Some empathetic responses but inconsistent, formulaic, rushed, or insufficiently attuned	No acknowledgment of emotions; dismissive, judgmental, or emotionally unsafe communication
4.2 Question Quality Did the student use effective questioning techniques? Are questions clinically accurate?	Good mix of open-ended and focused follow-up questions; starts broad, narrows logically; questions are clinically accurate and relevant; uses	Mostly closed yes/no questions but some open-ended ones; questions generally relevant but may lack clinical precision	Almost entirely closed yes/no questions; feels like an interrogation; questions may be clinically inaccurate or irrelevant

	appropriate sequencing		
4.3 Clarity / Professionalism Did the student communicate clearly and maintain professional boundaries?	Patient-friendly language; explains medical terms; professional tone; maintains boundaries; protects privacy; communicates limitations	Occasional unexplained jargon or slightly informal tone; mostly professional but some boundaries or limitations unclear	Excessive jargon; unprofessional; confusing language; breaches boundaries or overstates certainty
4.4 Handling Sensitive Topics Did the student approach sensitive areas with appropriate framing?	Approaches sensitive topics (substance use, mental health, sexual history) with appropriate framing and non-judgment ("I ask all my patients about…")	Asks about sensitive topics but bluntly or without framing	Avoids sensitive topics entirely or handles them inappropriately
4.5 Listening & Response to Cues NEW Did the student pick up on verbal cues, concerns, and changes in the patient's story?	Recognizes verbal cues, uncertainty, concerns, and changes in the patient's story; avoids interrupting; follows up on important information	Responds to obvious cues but overlooks subtler concerns or inconsistencies	Ignores, contradicts, or prematurely redirects important information
4.6 Clinical Reasoning & Synthesis NEW Did the student demonstrate sound clinical thinking during the encounter?	Identifies important findings; distinguishes relevant from irrelevant; recognizes red flags or	Recognizes some key findings but misses connections, alternatives, or uncertainty	Reasoning is unsupported, unsafe, internally inconsistent, or absent when required

	uncertainty; reasoning is clinically coherent and appropriately cautious		
Total Score	**16 items × 3 = 48 points**		

Note: A score of 1 in Clinical Reasoning & Synthesis (4.6) or Clarity/Professionalism (4.3) should trigger review even when the total is high.

Plus: Learning objective results (met/not met), top strengths with transcript citations, areas for improvement with transcript citations, and actionable suggestions.